Case study: Robin

Condition: **Spinal cord injury – T12 AIS C incomplete paraplegia**

52-year-old non-binary person · Inpatient rehabilitation unit, regional public hospital

Robin Taylor is a 52-year-old non-binary person (they/them) with **T12 AIS C incomplete paraplegia** following a fall from the roof of a house they were renovating five weeks ago. Robin had spinal fusion surgery in the acute hospital and transferred to the rehabilitation unit in Townsville two weeks ago. Robin is a self-employed carpenter and lives with their partner, Mel, and their daughter, Ava, in a lowset house with three steps at the front door.

Robin has made steady progress. Physiotherapy has documented that Robin transfers with standby assistance using a slide board and is "cleared for weekend home leave". Nursing notes from yesterday describe Robin as "independent with pressure care and in good spirits". Occupational therapy to date has focused on lower-body dressing, wheelchair skills and planning for the leave weekend. The medical team signed the home leave form this morning, and Robin's first weekend at home is planned to start at 2 pm today. Ava turns 18 tomorrow, and the family has planned a small party.

You have been asked to complete car transfer practice with Robin in the unit's car transfer bay before they leave. Robin is cheerful and full of plans. They tell you they have been "counting down the hours" and ask whether their new pressure-care cushion can go in the car or should stay on the wheelchair.

During the first transfer, you help Robin lift their left leg into the footwell and notice that the left calf looks noticeably bigger than the right. The skin is red and feels warm to touch. Robin says the leg has felt "a bit tight" since yesterday but that they "can't feel much down there anyway". As they settle into the car seat, Robin pauses, takes a sharp breath in and presses a hand to the right side of their chest. "Just a stitch. It catches when I breathe in deep."

Robin admits they have felt "a bit puffed" since this morning, even sitting still, and laughs it off. "It's nerves. First time out in five weeks, and I've got a teenager turning 18." You notice that Robin is breathing faster than usual, and their smartwatch shows a heart rate of 118. When you ask, they say they normally sit "in the seventies".

Robin then starts to talk about work. They have no idea whether they are covered by workers' compensation because they are self-employed. "If I can't get back on the tools, I don't know how we'll pay the mortgage." They ask whether you could write a letter about their injury for their bank, and whether they will ever be able to climb a ladder again.

There are no documented observations since 6 am. You do not know whether the leg has been reviewed by nursing or medical staff, or whether Robin's daily blood-thinning injections have been given as charted.

At 1.15 pm, Mel arrives early at the car bay with the leave medications from pharmacy. Mel has measured the front steps and asks whether you can quickly talk through a ramp and whether Robin will be able to drive with hand controls. Mel starts loading the wheelchair into the boot. Robin rubs their calf and says, "Let's just go. I'll put my feet up at home and I'll be right."

You are the occupational therapy student completing car transfer practice with Robin and your clinical supervisor is not currently in the room.

Present Robin's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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